

FINANCIAL PROFORMA

2-Year Nursing Value Model

Nursing

PREPARED FOR

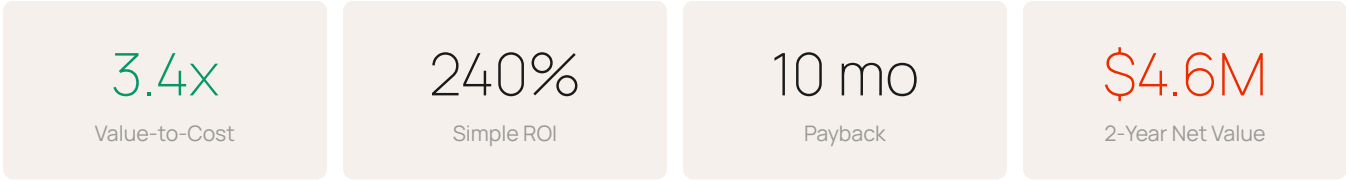
Corewell

PREPARED BY

John Haas · April 17, 2026

EXECUTIVE SUMMARY

The Investment Case at a Glance



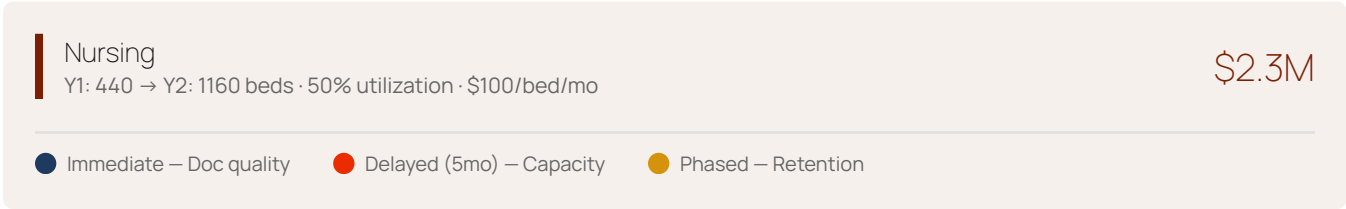
This 2-year partnership is projected to deliver \$4.6M in net organizational value — a 3.4x return on investment.

Across 440 beds over 2 years, the total investment is \$1.9M.

Value realization follows a deliberate phasing: documentation quality improvements begin after a 6-month implementation ramp, capacity and efficiency gains follow after an additional 5-month operational lag, and retention value phases in conservatively as clinician satisfaction compounds.

At full scale, the model projects \$5.7M in annual recurring value with payback at month 10.

KEY INPUTS



MODEL INPUTS

Key Assumptions & Configuration

Every number in this model traces back to specific inputs provided during configuration. This page documents the assumptions that drive the projections.

DEPLOYMENT PROFILE

Nursing		Per Provider/Month	
SCALE	UTILIZATION	PRICING	ANNUAL VALUE
440 staffed beds 440 nurse FTEs	Y1: 50% Y2: 50% 27.9 min saved/encounter	\$100/mo	\$2.3M

IMPLEMENTATION & PHASING

CONTRACT TERM	IMPLEMENTATION RAMP	OPERATIONAL LAG
2-Year	6 months	5 months
RETENTION PHASING	VALUE ONSET	
Y1 35%	Y2 75%	Doc quality: immediate after ramp Capacity & efficiency: +5mo delay Retention: phased per schedule above

VALUE DRIVERS

How the Value Breaks Down

Each driver below represents a specific, measurable improvement. Together, they form the basis of the projected return.

HAPI Risk Reduction IMMEDIATE	\$587K
HAPI risk reduction through improved documentation = \$587K/year	
Fall Risk Visibility IMMEDIATE	\$329K
Fall risk visibility gap closure = \$329K/year	
OT Reduction DELAYED (5MO)	\$1.0M
660 nurses × 1 OT hrs/wk × 52 wks = 34,320 OT hrs/yr 34,320 × 40% reduction = 13,728 hrs eliminated 13,728 × \$75/hr = \$1.0M/year	
Nurse Retention PHASED	\$401K
440 FTEs × 18% turnover × 40% burnout-related × typical impact × \$56,300 replacement = \$401K/year	

CATEGORY SUBTOTALS

\$2.7M Documentation Quality 41%	\$3.0M Capacity & Efficiency 47%	\$792K Retention 12%
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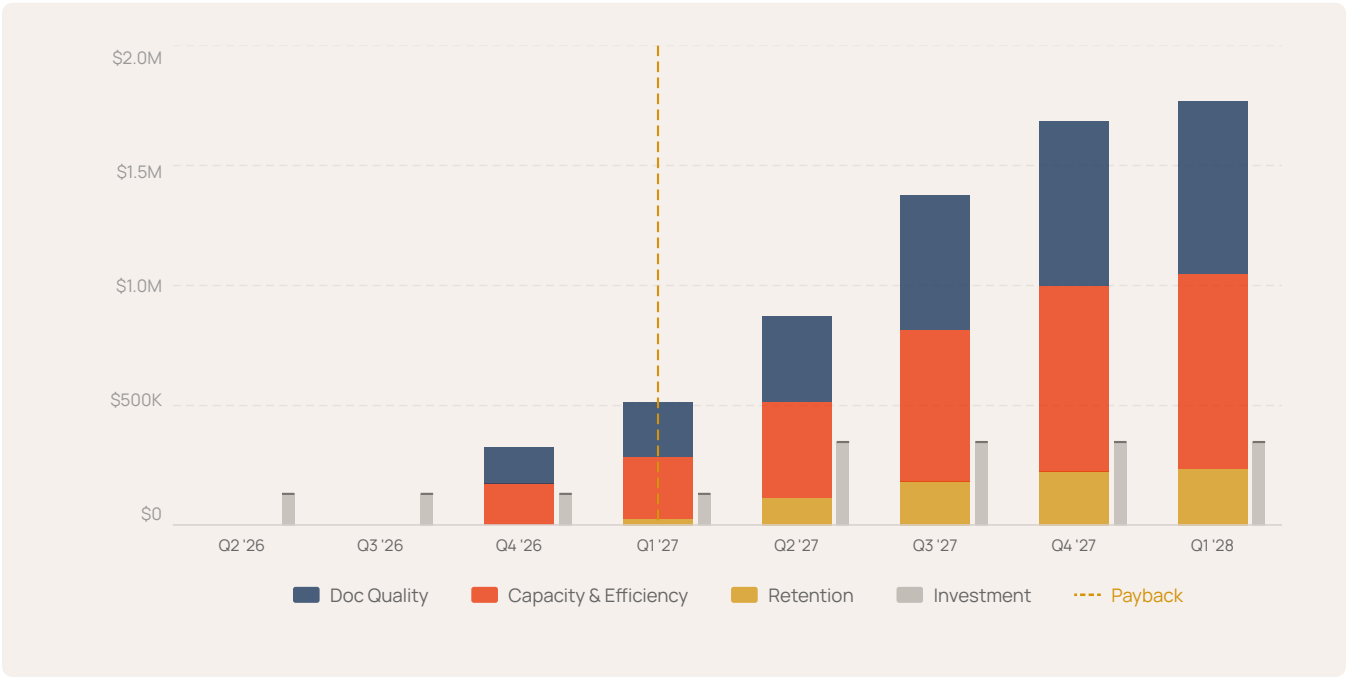
WORKFORCE IMPACT

23,938 hours/year returned to clinical care	1.0 hours/week per bed	7 days/year not at a keyboard
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2-YEAR PROJECTION

How Value Builds Over Time

Value builds progressively as the deployment matures. Documentation quality gains (navy) appear first, capacity and efficiency improvements (red) follow after a 5-month operational lag, and retention value (gold) compounds over the contract term.



	2026	2027	TOTAL
Nursing	\$840K	\$5.7M	\$6.5M
Total Value	\$840K	\$5.7M	\$6.5M
Doc Quality (immediate)	\$382K	\$2.3M	\$2.7M
Capacity & Efficiency (5mo delay)	\$429K	\$2.6M	\$3.0M
Retention (phased)	\$29K	\$763K	\$792K
Licensed Providers	440	1,160	1,160
Actively Documenting	174	558	558
Investment	(\$528K)	(\$1.4M)	(\$1.9M)
Net Value	\$312K	\$4.3M	\$4.6M

Payback period reflects the month in which cumulative net value turns positive, accounting for the 6-month implementation ramp and subscription costs from day one. Actual time to value may vary based on training completion, workflow integration, and provider adoption speed.

YEAR-BY-YEAR OUTLOOK

How the Deployment Unfolds

Each year of the partnership has a distinct character. Value compounds as adoption deepens, retention effects materialize, and the organization operationalizes freed-up capacity.

23,938 annual hours returned to clinical care

440 beds at full scale

\$5.7M annual run-rate value at maturity

YEAR 1 (2026)

Building the Foundation with 440 beds

\$840K
projected value

The 6-month implementation ramp establishes workflows and adoption. Value begins with documentation quality, capacity and efficiency (after a 5-month operational lag), early retention effects. Year 1 projects \$840K in organizational value.

LICENSED	ACTIVE	ADOPTION RATE	INVESTMENT	NET
440	174	40%	\$528K	\$312K

YEAR 2 (2027)

Scaling to 1,160 beds

\$5.7M
projected value

Expanded deployment broadens the value base. Retention value begins to materialize as clinician satisfaction compounds. Year 2 projects \$5.7M in organizational value.

LICENSED	ACTIVE	ADOPTION RATE	INVESTMENT	NET
1,160	558	48%	\$1.4M	\$4.3M

VALUE COMPOSITION



STRATEGIC OBSERVATION

This Nursing deployment presents a compelling financial case, with capacity and efficiency representing 47% of total value. A single-setting model provides a focused proof of value – once outcomes are validated, this framework extends naturally to additional care settings. Payback within the first year reinforces the near-term financial case.

SCENARIO ANALYSIS

Confidence Range & Cost of Delay

Robust financial planning accounts for variability. This section brackets the range of likely outcomes and quantifies the opportunity cost of deferred implementation.

SENSITIVITY ANALYSIS



COST OF INACTION



MODEL CONFIDENCE



METHODOLOGY & ASSUMPTIONS

Modeling Framework & Assumptions

YOUR INPUTS

Nursing

Y1: 440 → Y2: 1160 beds

\$100/bed/mo

440 staffed beds · 440 nurse FTEs · 50% utilization · 27.9 min
saved/encounter · OT: 40% reduction · \$75/hr · Retention: 18%
turnover · \$56,300 replacement

CALCULATION METHOD

Contract term: 2-Year (24 months)

Implementation ramp: 6 months (gradual onset)

Utilization targets: 55% Y1, 75% Y2 (Nursing: 50/60%)

Provider expansion: Per-year allocation

Onset timing: Immediate / 5mo delay / phased

Retention phasing: 35% Y1, 75% Y2

RETURN METHODOLOGY

Value-to-Cost: total value / total cost.

Simple ROI: net value / total cost.

Payback: month cumulative net value turns positive.

DATA SOURCES

- Capacity & efficiency: Informed by Abridge health system deployment experience and published time-motion research
- Industry benchmarks: MGMA, CMS, proprietary health system datasets
- Conservative design: Where uncertainty exists, conservative assumptions applied
- Retention: Delayed-onset, phased conservatively over 2 years

LIMITATIONS

This proforma is for financial planning purposes only. Projections are modeled estimates based on user-provided inputs and published benchmarks. Actual results will depend on implementation quality, organizational adoption, and operational factors unique to your environment. These projections do not constitute a guarantee of financial outcomes. We encourage validation of every assumption against organization-specific data.

ABRIDGE

Prepared for

Corewell

April 17, 2026

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2-Year Financial Proforma · Nursing

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